USER INFORMATION
READ CAREFULLY

INFUSOL® D30
DEXTROSE 30% w/v INTRAVENOUS INFUSION BP

Composition:
The Solution Contains:

Dextrose Anhydrous	300 g/L
Osmolarity:	1665 mOsm/L
Caloric Value:	5024 kJ/L = 1200 kcal/L

Product Description:
Dextrose 30% w/v Intravenous Infusion BP. The solution is a colourless or not more than faintly straw coloured solution.

Pharmacodynamics:
Glucose provides energy. Glucose when given intravenously with water metabolises into glycogen leaving water to be absorbed to correct fluid imbalance. Glucose increases the movements of electrolytes across the semi-permeable membrane.

Pharmacokinetics:
Glucose is rapidly absorbed from the gastrointestinal tract. It is metabolised to carbon dioxide and water with the release of approx. 4 calories/gram of energy. It is readily metabolised. It may deaccelerate body protein and nitrogen losses, promote glycogen deposition and decrease/prevent ketosis in sufficient dose. As the product is administered through intravenous route it is 100% bioavailable.

Indication:
Parenteral Nutrition.
Hypoglycaemia.
High caloric carbohydrate therapy, especially when fluid intake is limited.

Recommended Dose:
Adult patients.
According to individual requirements:
Dextrose 30% : Up to 23mL/kg of body weight and per day.
Up to 1.7mL/kg body weight and per hour.

Pediatric patients.
Mean requirements/kg of body weight and per day.

1st year of life	: 8 – 15g dextrose
2nd year of life	: 12 – 15g dextrose
3rd-5th year of life	: 12g dextrose
6th-10th year of life	: 10g dextrose

Route of administration: Intravenous (IV)
Glucose solution with a concentration greater than 5% are hypertonic and should be administered by slow intravenous infusion via a central vein.

Contraindications:
Diabetes mellitus (with the exception of hypoglycaemia conditions)
Glucose intolerance, hypotonic dehydration if lacking electrolytes are not replaced.
Over hydration.
Hypokalemia.
Hyperosmolar coma
Acidosis

Warnings and Precautions:
Blood glucose, serum electrolytes and water balance should be monitored regularly. Electrolytes are to be supplemented as required.
The compatibility of any additives to this solutions should be checked before use.
Dextrose solutions should not be administered through the same infusion set through which also blood has or may be given because of the risk of pseudoagglutination.
Single dose container. Discard all unused contents. Do not use if leakage is detected. If any visible solid particles, growth or turbidity appears during storage, the product should not be used.

Statement on usage during pregnancy and lactation:
Intravenous glucose may result in foetal insulin production, with an associated risk of rebound hypoglycaemia in the neonate. Infusions of glucose administered during Caesarean section and labour should not exceed 5-10g glucose/hour.

Adverse Effects/ Undesirable Effects:
Dextrose Injection and particularly hypertonic dextrose injection may have low pH, and such solution when injected may irritate the venous intima near the site of the injection to causing thrombophlebitis.
Hyperglycaemia and renal losses may occur in case of reduced glucose tolerance.These manifestations are normally prevented by reducing the dosage and/or giving insulin.
Enhanced bilirubin and lactate levels may be found if the recommended dosage is exceeded.

Overdose and Treatment:
Dextrose solution I.V. can cause fluid overload resulting in dilution of serum electrolyte concentrations, overhydration, congested states or pulmonary oedema.
Hyperglycaemia and glucosuria may be functions of rate of administration or metabolic insufficiency. To minimise these condition, slow infusion rate, monitor blood and urine glucose; if necessary, stop administration of dextrose or /and administer insulin.

Storage Condition:
Do not store above 30°C.

Shelf Life:
3 years from manufacturing date in the proposed storage condition.
Do not use after expiry.

Dosage forms and packaging available:
500mL x 20 LDPE plastic bottle per carton
500mL x 20 PVC Medical plastic collapsible bag per carton
10mL x 20 LDPE plastic ampoule per Box

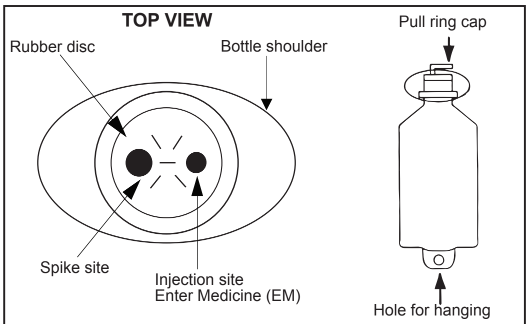
Manufacturer / Product Registration Holder:
AIN MEDICARE SDN.BHD.
Lot 4933, 4934 & PT2464, Jalan 6/44, Kaw. Perindustrian Pengkalan Chepa 2, 16100 Kota Bharu, Kelantan, MALAYSIA.

HANDLING INSTRUCTION

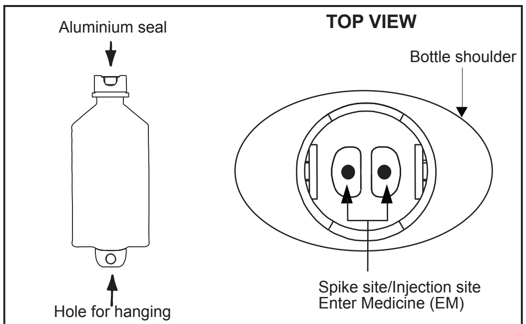
AIN MEDICARE INTRAVENOUS INFUSION FLUID IN LOW DENSITY POLYETHYLENE (LDPE) BOTTLE

The bottle is made from injectable grade polyethylene.

PARTS OF THE PLASTIC BOTTLE



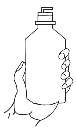
Design 1: Pull Ring Cap



Design 2: Twin Port Cap (Aluminum Seal)

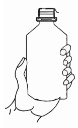
Top view after pull ring cap is removed. Please refer whichever applicable design.

INSTRUCTION FOR THE USAGE OF IV ADMINISTRATION SET & ADDITION OF DRUG

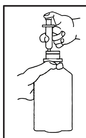


Design 1

1. Before use check for any solid particle, growth, turbidity or leakage. Discard the product if particle, growth, turbidity or leakage is found.

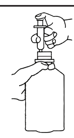


Design 2

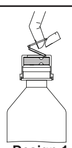


Design 1

3. Additives may be injected through injection site (Refer to Design 1 or 2 whichever applicable) on the rubber. Mix the solution thoroughly.



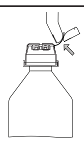
Design 2



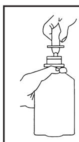
Design 1

2. Design 1: Pull the cap's ring until completely removed.

   Design 2: Pull the aluminum seal until completely removed by centre line.



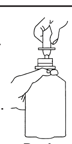
Design 2



Design 1

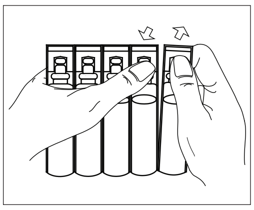
4. Hold bottle down firmly and insert a spike of administration set into the spike site for Design 1 or for Design 2, pull the other aluminium seal until completely removed before inserting a spike of administration set.

(Spike site is meant for spiking of the administration set and is only for single use)

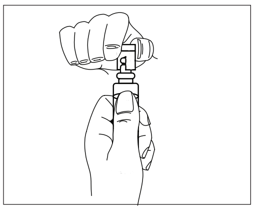


Design 2

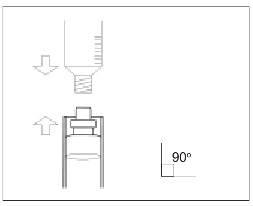
AIN MEDICARE INJECTION SOLUTION IN LOW DENSITY POLYETHYLENE (LDPE) AMPOULE



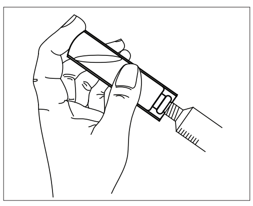
1. Detach ampoule along parting line



2. Twist off tab



3. Attach luer lock syringe, luer slip syringe or hypodermic needle vertically to the ampoule.



4. Draw out injection fluid using luer lock syringe, luer slip syringe or hypodermic needle. Ensure the connection is tightly secured for effective aspiration of the liquid.

Date of Revision: 01/11/2019